

problem in the baby. The condition increases the risk of premature labor and cord prolapse. You will be carefully monitored and may be advised to rest. In severe cases, the fluid may be drained.

Oligohydramnios Too little amniotic fluid may be due to early rupture of the membranes; placental problems; fetal abnormalities; or problems with the baby's growth (see [Assessing fetal growth and well-being](#)). A reduction of amniotic fluid is most likely toward the end of pregnancy. If an ultrasound scan confirms that levels are low and there are concerns about the baby's growth, early delivery may be advised.

Placental insufficiency occurs when the placenta isn't functioning well enough. Some signs are a reduction in the amount of fluid around the baby, a decrease in the growth of the baby's abdomen and hence their weight, and abnormalities in an ultrasound (see [Measuring growth with ultrasound](#)).

Causes Placental insufficiency is more common in women with preeclampsia, those who have an underlying medical problem, and in women who smoke. It also occurs more often in babies with a chromosomal abnormality or a structural congenital abnormality.

What might be done It is usually detected by the doctor noticing that your uterus measures smaller than expected and referring you for an ultrasound. If the baby isn't growing well, the doctor will watch you closely and may recommend bed rest for some or the rest of your pregnancy. If the placenta is working poorly, your baby may not be able to cope with labor; a Cesarean may be recommended.

Bleeding in late pregnancy If bleeding occurs, you and the baby need to be assessed immediately. Bleeding caused by a problem with the placenta can be a serious threat to your baby.

Causes The most serious causes are placenta previa (see [Low-lying placenta](#)) and placental abruption. With a placenta previa, the placenta lies low in the uterus and bleeding is typically painless. In a placental abruption, the placenta starts to separate from the uterus, which can be very painful as the blood can be trapped inside the uterus. Both of these can be life-threatening to your baby. Other, less-dangerous causes include cervical friability, or bleeding from the cervix itself, especially after intercourse, or from a cervical polyp.

What might be done If bleeding is mild, your doctor may recommend bed rest. If your doctor thinks you have abruption, they may send you to the hospital for observation. Depending on how far along you are, you may be given corticosteroids to help the baby's lungs mature in case an early delivery is necessary. Your doctor may send you home when the bleeding stops. Sometimes, delivering the baby is the safest option. If bleeding is heavy or painful, or the baby is distressed, an emergency Cesarean and a blood transfusion may be needed.

Preeclampsia Preeclampsia (also known as toxemia) is a pregnancy-induced condition characterized by high blood pressure, protein in the urine, and edema (swelling). Occasionally women have symptoms such as headaches, flashing lights, abdominal pain, or nausea. If left untreated, it can lead to eclampsia, an extremely serious condition that causes convulsions and coma. If diagnosed with preeclampsia, your pregnancy will be watched very closely and a decision will be made as to the best time for delivery. About 8 percent of women have problems with high blood pressure in pregnancy.

Causes Preeclampsia is more common in multiple pregnancies; in very young and older mothers; in women with preexisting high blood pressure or kidney disease; in women who've had severe preeclampsia before, necessitating delivery by 32 weeks; and in women who've had an egg donation.

What might be done Although the only cure for preeclampsia is delivery of the baby, the baby may need longer to mature in the uterus. The mother and baby will be closely